



TYPES OF PELVIC FLOOR DISORDERS

Pelvic organ prolapse

- Uterine prolapse
- Post-hysterectomy vault prolapse
- Cystocele
- Rectocele



Bladder control problems

- Frequency
- Urgency
- Urinary incontinence
 - Stress incontinence
 - Urgency incontinence
 - Overflow incontinence

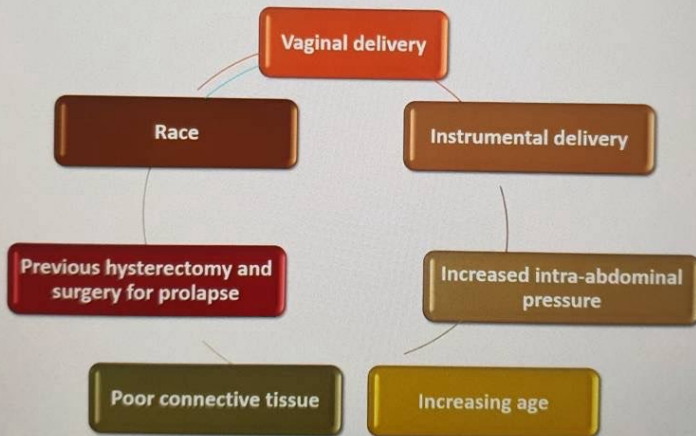


Bowel control problems

- Fecal urgency
- Fecal incontinence
- Obstetric anal sphincter injuries



RISK FACTORS FOR PELVIC FLOOR DISORDERS



TREATMENTS FOR PELVIC FLOOR DISORDERS

Lifestyle changes

- **Limit foods and drinks that stimulate the bladder**
 - e.g., caffeinated beverages, carbonated beverages, citrus fruits and drinks, artificial sweeteners, and alcoholic beverages
- **Eat a high-fiber diet**
 - To help digest food and prevent constipation
 - Found in fruits, vegetables, beans and lentils and whole grains
- **Lose weight to relieve pressure on pelvic organs**

TREATMENTS FOR PELVIC FLOOR DISORDERS

Nonsurgical treatments

- **Bladder training**
 - Techniques to suppress the urge to urinate
 - Go to toilet at a specific interval and slowly increases to every 2.5 to 3 hours.
- **Pelvic floor muscle training (PFMT)**
 - Improve symptoms of incontinence and POP
- **Medicine**
 - To treat certain bladder control problems
- **Vaginal pessary**
 - For POP and urinary incontinence



TREATMENTS FOR PELVIC FLOOR DISORDERS

Surgical treatments

For pelvic organ prolapse

- Involves repairing the prolapse and attempting to restore a well-supported anatomy.

For bladder control problems

- Aims to treat problems with urinary leakage
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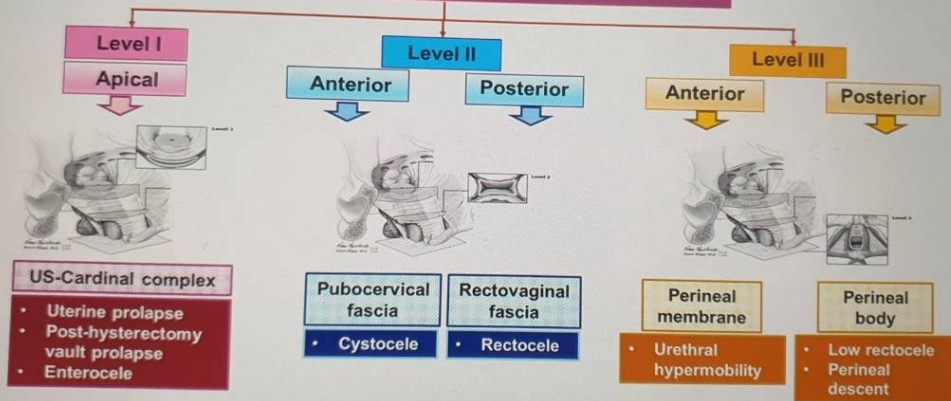
For bowel control problems

- To repair a damaged anal sphincter muscle
- To inject medications into the sphincter
- To implant a stimulator for the nerves that control the bowel function.



PATHOPHYSIOLOGY OF POP

Attenuation or damage of endopelvic fascia

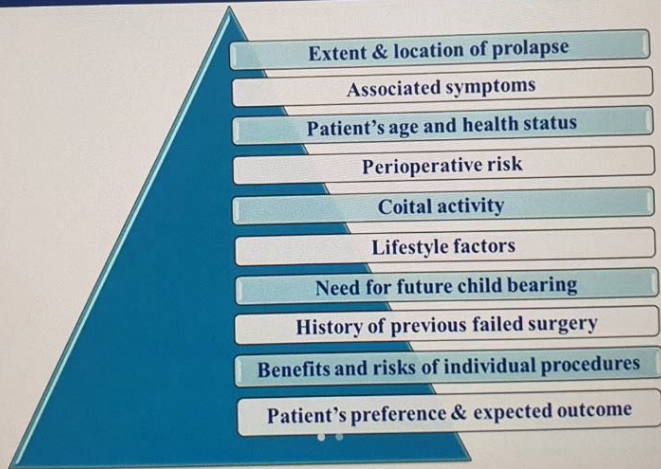


PELVIC RECONSTRUCTIVE SURGERY FOR POP

Route / Type of surgery		Apical		Anterior	Posterior
		Vault suspension / No uterus	Uterine preservation		
Vaginal	Restorative	- McCall culdoplasty - High uterosacral ligament suspension - Sacrospinous ligament suspension - Iliococcygeus fascial suspension	- Uterosacral ligament hysteropexy - Sacrospinous hysteropexy	- Anterior colporrhaphy - Paravaginal repair	- Posterior colporrhaphy +/- perineorrhaphy
	Compensatory	- Sacrospinous mesh colpexy	- Sacrospinous mesh hysteropexy	- Anterior vaginal mesh repair	- Posterior vaginal mesh repair
	Obliterative	- Total colpocleisis	- Partial colpocleisis		
Abdominal	Restorative	- Uterosacral ligament suspension	- Uterosacral ligament hysteropexy	- Paravaginal repair	
	Compensatory	- Sacrocolpopexy / Sacrocervicopexy	- Sacrohysteropexy	- Sacrocolpopexy	- Sacrocolpopexy
Laparoscopic or Robotic	Restorative	- Uterosacral ligament suspension	- Uterosacral ligament hysteropexy - Round ligament hysteropexy	- Paravaginal repair	
	Compensatory	- Sacrocolpopexy / Sacrocervicopexy - Lateral suspension for vault prolapse - Pectopexy	- Sacrohysteropexy - Uterus-preserving lateral suspension - Pectopexy	- Sacrocolpopexy - Lateral suspension - Pectopexy	- Sacrocolpopexy - Sacral colpoperineopexy - Lateral suspension - Pectopexy



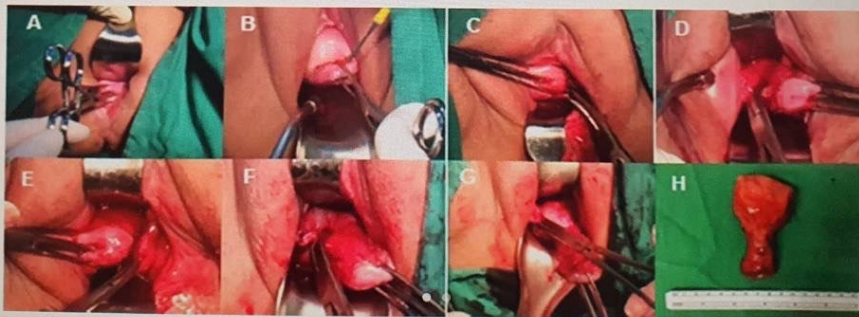
CONSIDERATION FOR POP SURGERY



PELVIC RECONSTRUCTIVE SURGERY FOR POP

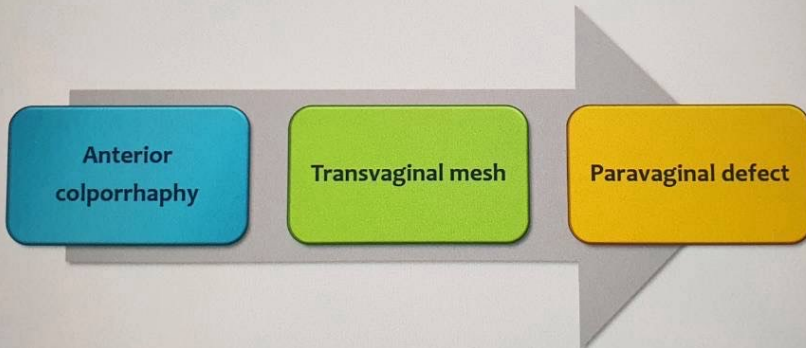
Vaginal hysterectomy

- Vaginal hysterectomy is the approach of choice whenever feasible.
- According to Cochrane review, vaginal route when compared with all other routes for hysterectomy yields better outcomes and fewer complications.



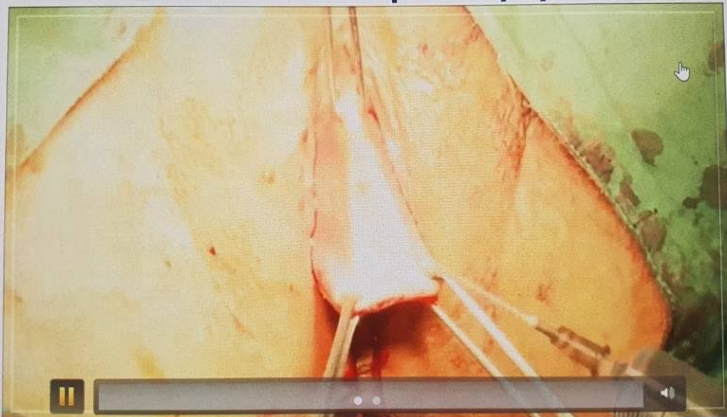
PELVIC RECONSTRUCTIVE SURGERY FOR POP

Surgery for anterior vaginal wall prolapse



PELVIC RECONSTRUCTIVE SURGERY FOR POP

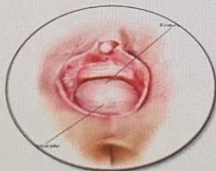
Anterior colporrhaphy



PELVIC RECONSTRUCTIVE SURGERY FOR POP

Surgery for posterior vaginal wall prolapse

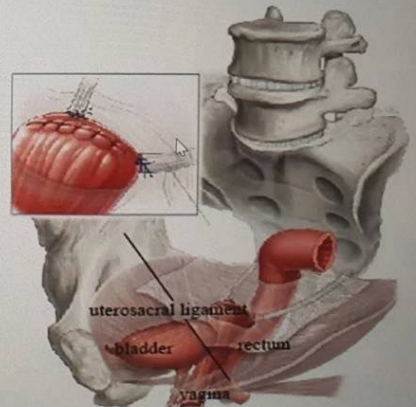
- Each level of rectovaginal fascial defect results in herniation of posterior vaginal wall with anterior rectal wall in direct apposition to vaginal epithelium
 - Level I – Enterocele
 - Level II – High rectocele
 - Level III – Low rectocele
- Defect of perineal membrane & detachment of rectovaginal fascia from perineal body lead to
 - Low rectocele
 - Perineal descent



PELVIC RECONSTRUCTIVE SURGERY FOR POP

High uterosacral ligament suspension

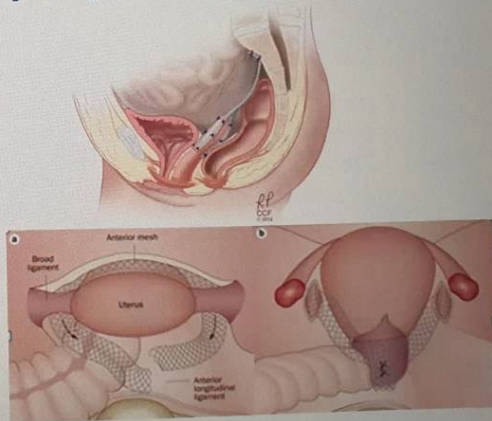
- Suspension of vaginal apex to remnants of uterosacral ligaments at level of ischial spines.
- Provides consistent support with high success rates 89-98%.
- Maintain vaginal axis in midline with excellent restoration of vaginal length
- Intraoperative ureteric injury 1-11%
- Laparoscopic USLS has comparable clinical outcomes, with similar rates of complications and prolapse recurrence to the traditional vaginal approach.



PELVIC RECONSTRUCTIVE SURGERY FOR POP

Abdominal sacrocolpo/hysteropexy

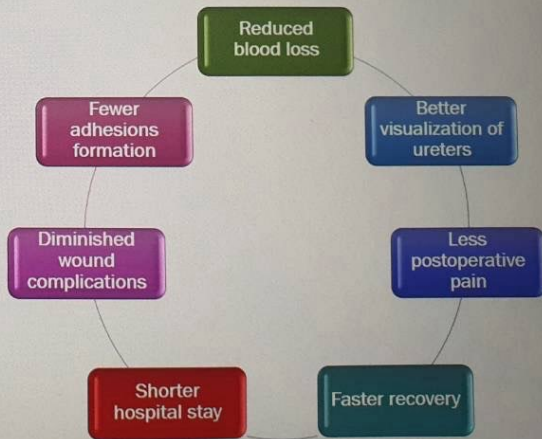
- A surgical technique used to treat apical compartment prolapse and some cases of multicompartment prolapse.
- Vaginal vault (or the cervix in cases of supracervical hysterectomy) is suspended to the presacral ligament, typically polypropylene mesh.



PELVIC RECONSTRUCTIVE SURGERY FOR POP

Open vs Laparoscopy

- Laparoscopic pelvic reconstruction is gaining growing attention, primarily due to less invasiveness and high effectiveness resulting from easier access to pelvic supportive structures.



SURGERY FOR STRESS INCONTINENCE

- In patients undergoing surgery for stress urinary incontinence, physicians may perform any of the incontinence procedures
 - Midurethral sling
 - Pubovaginal sling
 - Burch colposuspension

Retropubic midurethral sling



REPAIR OF OBSTETRIC ANAL SPHINCTER INJURIES

OASIS repair



CONCLUSION

- The objective of surgical Rx for pelvic floor disorders is anatomical restoration that subsequently leads to normalized function of pelvic organs.
- Given the etiological variability, a wider range of procedures is required to allow a tailored approach to the individual patient.
- Traditional vaginal procedures and transvaginal mesh are being replaced by laparoscopic techniques that offer anticipated benefits in reduced recurrence and complication rates.
- By offering several treatment alternatives, clinician can help to ensure that the largest number of symptomatic women achieves their personal goals.